

Undergraduate Medicine

Dermatology Study Pack

CharaNas Medicine — Specialty Notes

Expanded notes with original schematic figures for revision. These are educational drawings inspired by standard undergraduate teaching themes, not copied textbook plates. Always follow your faculty curriculum and latest guidelines.

Section	Focus
1	Skin structure & lesion language
2	Infection & infestation
3	Inflammatory dermatoses
4	Skin cancer vigilance
5	Checklist & books
6	Quick pearls from these notes
7	Recommended book sources

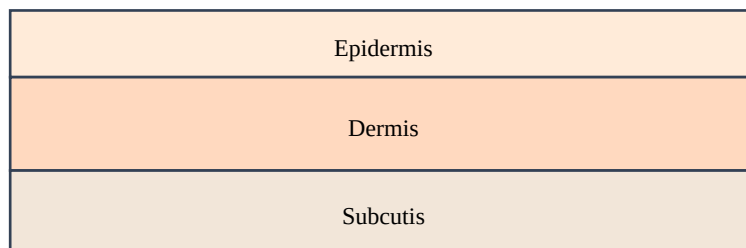
1. Skin structure & lesion language

Learn lesion morphology: macule, papule, plaque, vesicle, bulla, pustule, nodule, ulcer. Distribution and arrangement (dermatomal, flexural, extensor) are diagnostic clues.

Epidermis vs dermis vs subcutis depth helps distinguish superficial infections from deeper soft-tissue infection.

Always ask about drugs, atopy, occupation, travel, contacts, immunosuppression, and systemic symptoms.

Figure: Skin layers (schematic)



Depth matters: impetigo (epidermis) vs cellulitis (deeper soft tissue)

Schematic figure for learning — verify details in your recommended textbooks.

Revision prompts

- Explain this section aloud in 60 seconds as if teaching a junior student about dermatology.
- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

2. Infection & infestation

Impetigo: honey-colored crusts, common in children, contagious. Cellulitis: spreading erythema, warmth, tenderness — mark borders and treat per guidelines.

Abscess usually needs drainage. Necrotizing infection: pain out of proportion, rapid progression, systemic toxicity — surgical emergency.

Scabies: intense nocturnal itch, finger webs/flexures; treat index case and close contacts; wash linens.

Revision prompts

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- List 3 red flags that would make you escalate care urgently.
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3. Inflammatory dermatoses

Eczema/atopic dermatitis: itchy flexural inflammation; emollients are foundation therapy. Psoriasis: well-demarcated plaques with silvery scale, extensor surfaces, Auspitz sign.

Urticaria vs anaphylaxis: look for airway/breathing/circulation involvement — anaphylaxis is emergency adrenaline territory.

Drug eruptions range from simple exanthem to SJS/TEN — stop culprit drug early and escalate severe reactions.

Revision prompts

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- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

4. Skin cancer vigilance

ABCDE for melanoma: Asymmetry, Border irregularity, Color variation, Diameter, Evolving. Suspicious lesions need urgent specialist excision pathways.

Basal cell carcinoma: pearly nodule with telangiectasia common. Squamous cell carcinoma: scaly/ulcerated nodule; higher metastasis risk than BCC.

Sun protection counseling is preventive medicine.

Revision prompts

- Explain this section aloud in 60 seconds as if teaching a junior student about dermatology.
- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

5. Checklist & books

Checklist: full skin exam when relevant, lymph nodes for suspicious lesions, photograph/monitor, biopsy appropriately, infection control for contagious rashes.

Books: Rook; Fitzpatrick; Habif Clinical Dermatology.

Revision prompts

- Explain this section aloud in 60 seconds as if teaching a junior student about dermatology.
- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

6. Quick pearls

High-yield reminders packaged with this specialty in CharaNas:

1. ABCDE for melanoma; urgent specialist referral for suspicious lesions.
2. Impetigo: honey-colored crusts — contagious.
3. Psoriasis: silvery scale, Auspitz, extensor surfaces common.
4. Scabies: nocturnal itch, finger webs — treat patient + contacts.
5. Cellulitis vs abscess: antibiotics ± drainage if collection.
 - Link symptoms → anatomy/physiology → investigation → first management step.
 - Write one OSCE-style explanation for a common presentation in this specialty.
 - After reading, do the Short MCQ and Case-based Question for active recall.

7. Recommended book sources

Standard undergraduate references commonly used internationally. Use the edition your college recommends. Figures in commercial textbooks are copyrighted; use library/licensed access for full plates and photographs.

#	Reference
1	Rook's Textbook of Dermatology
2	Fitzpatrick's Dermatology
3	Habif's Clinical Dermatology

How to study with this PDF: read one section → sketch the figure from memory → answer the MCQs/cases → review mistakes → revisit the matching section.

Disclaimer: educational aid only; not a substitute for clinical guidelines, faculty teaching, or licensed textbooks.